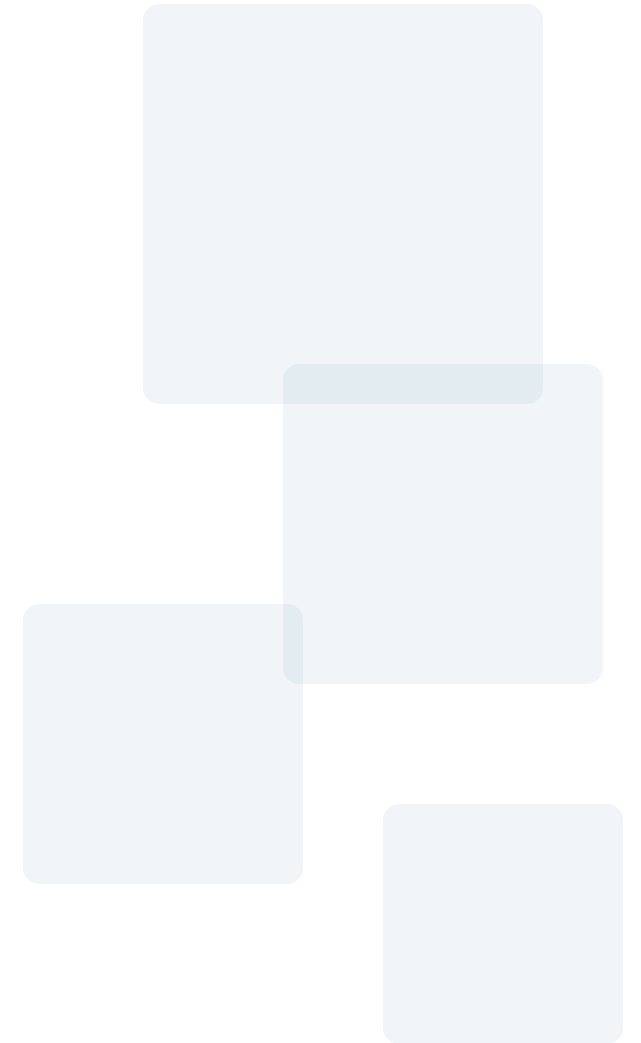

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

A data-driven overview for policy analysts and program planners · Global data, 2023



Hundreds of Millions Affected: Lifetime Major Depression Risk

1 in 3 women and 1 in 5 men will experience major depression — mental health conditions span from common disorders to severe illness.



1 in 3

Women

will experience major depression
in their lifetime



1 in 5

Men

will experience major depression
in their lifetime

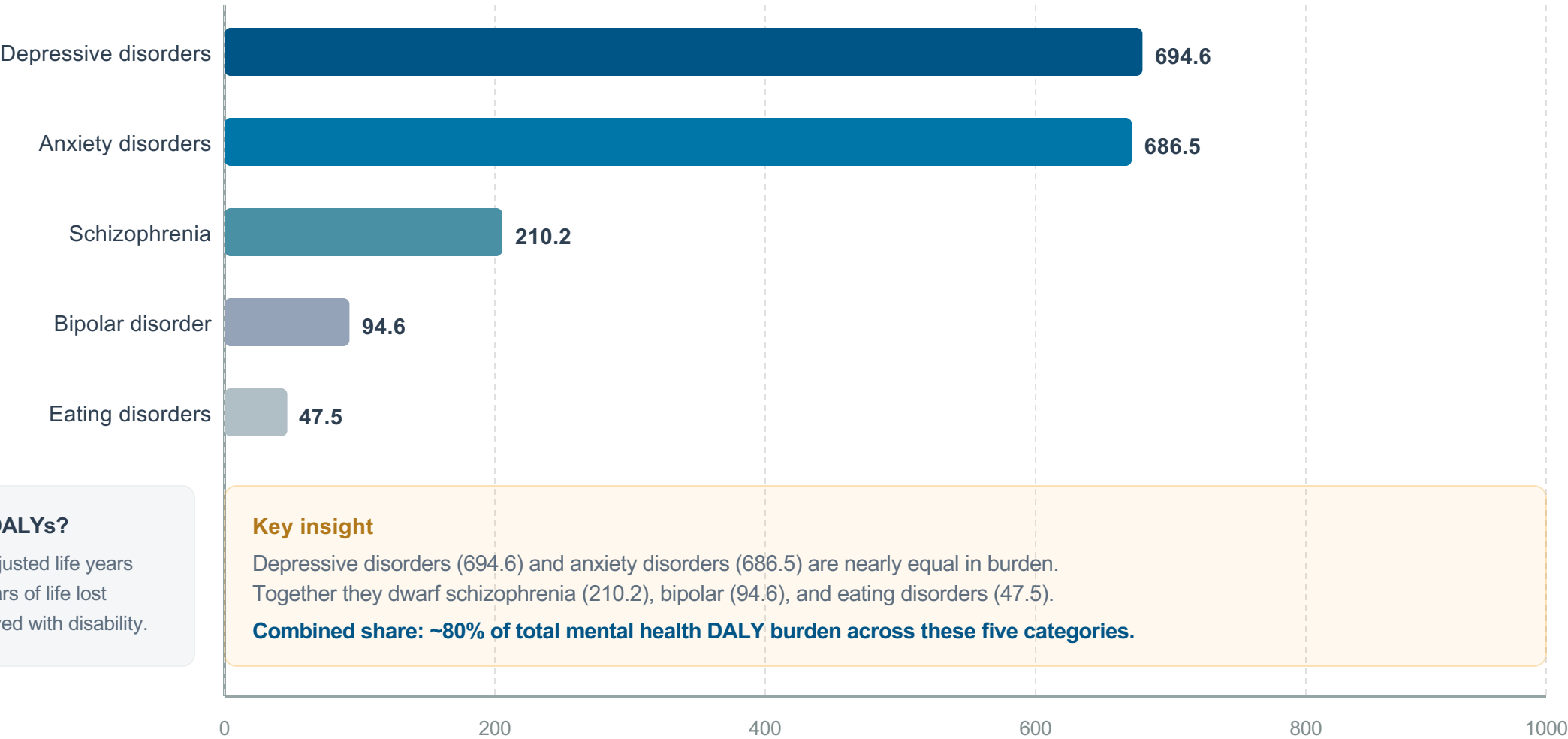
Why this matters for policy

Hundreds of millions of people suffer from mental health conditions each year. Conditions range from common (depression, anxiety) to less common but severe (schizophrenia, bipolar disorder). Poor mental health affects well-being, work capacity, and relationships with friends, family, and community.

Many more experience conditions over their lifetimes — prevalence is substantial.

Depression and Anxiety Dominate Disease Burden

Depressive and anxiety disorders together account for ~80% of the total mental health DALY burden across these five categories.



What are DALYs?

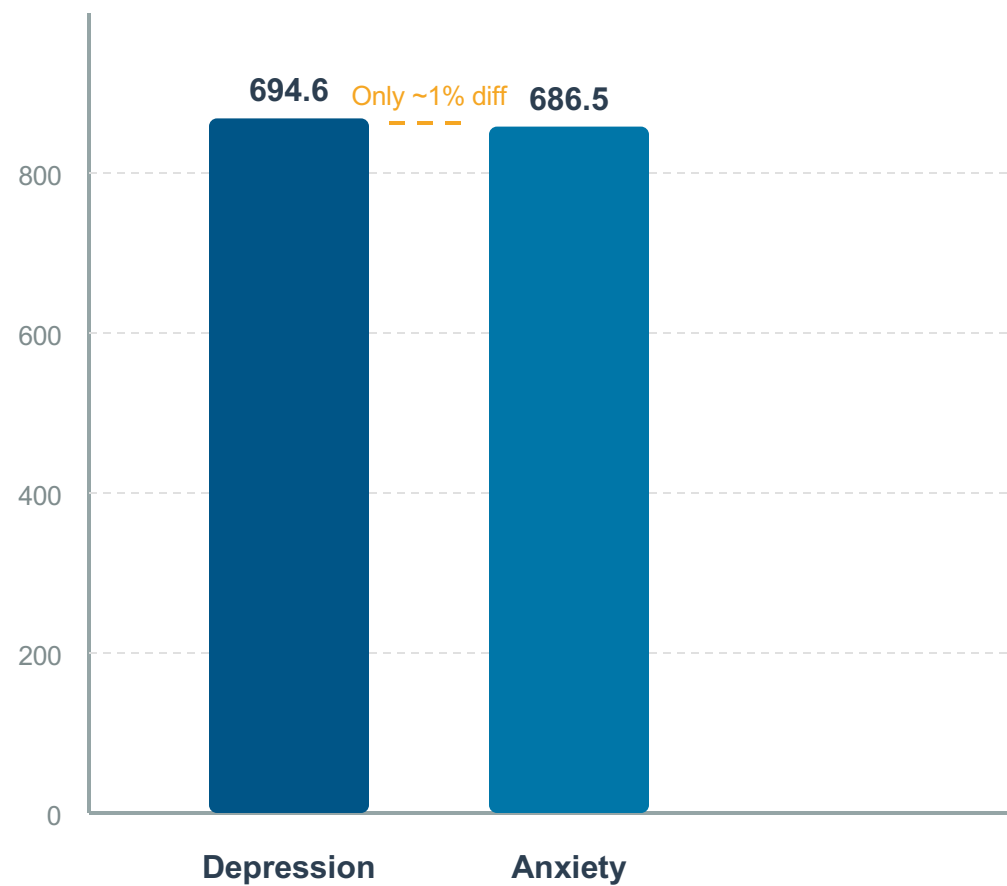
Disability-adjusted life years combine years of life lost and years lived with disability.

Key insight

Depressive disorders (694.6) and anxiety disorders (686.5) are nearly equal in burden. Together they dwarf schizophrenia (210.2), bipolar (94.6), and eating disorders (47.5).
Combined share: ~80% of total mental health DALY burden across these five categories.

Depression and Anxiety Carry Nearly Equal Disease Burden

The near-parity in DALYs means policy must address both depression and anxiety with equal urgency.



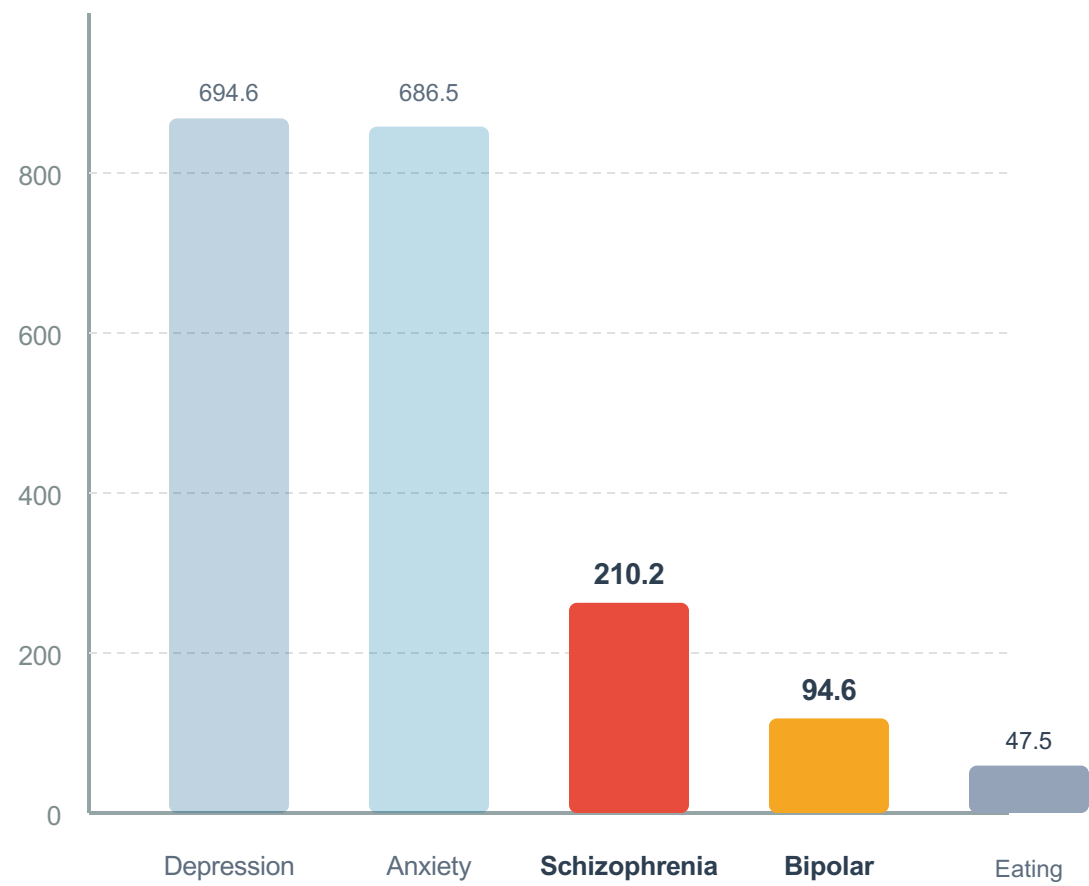
Key Insights

- Near-perfect parity**
694.6 vs 686.5 DALYs per 100K — a difference of only 8.1
- Overwhelming majority**
Together they account for ~80% of total burden across these 5 categories
- Policy implication**
Scaling interventions for both must be a simultaneous priority

Combined DALYs: 1,381.1 per 100,000 people

Schizophrenia and Bipolar: Lower Prevalence, Severe Impact

While population-level burden is lower than depression/anxiety, individual severity and disability are extremely high.



Severe Conditions in Context

Schizophrenia: 210.2 DALYs per 100K
Reflects severe disability per individual. While fewer people are affected, the condition imposes a disproportionately high burden on those who have it.

Bipolar disorder: 94.6 DALYs per 100K
A smaller but meaningful share. Individual episodes can be highly disabling and require long-term specialized management.

Policy implication
Specialized care infrastructure for severe conditions must be maintained even as common-disorder interventions are scaled.

Severe conditions + eating disorders = 352.3 DALYs per 100K combined

Treatment Is Available but Often Lacking or Poor Quality

Mental illnesses are treatable, yet stigma, poor access, and inadequate quality create a large treatment gap worldwide.



Stigma

Many people are uncomfortable disclosing symptoms to professionals or people they know.



Access

Treatment is often lacking, especially in low- and middle-income countries.



Quality

Even where treatment exists, it may be of poor quality or not evidence-based.



Underreporting

Discomfort with disclosure makes it difficult to estimate true prevalence.

The true burden is likely underestimated

Good data is essential to understand these conditions — how, when, and why they occur, how many people are affected, and how they can be treated effectively and safely.

Actual prevalence may be higher than reported figures suggest.

How People Cope: From Medication to Faith to Family

Coping strategies vary widely across countries and cultures — policy must account for these differences.



Prescribed Medication

Common in higher-income settings where healthcare infrastructure is more robust.

Clinical approach



Spiritual Activities

Religious or spiritual practices are prevalent in many regions worldwide.

Cultural approach



Talking to Friends / Family

A widely reported coping mechanism across all income levels and cultures.

Social approach

Policy implication: One size does not fit all

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Interventions must account for these differences.

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Gap

Policy should scale evidence-based interventions for common disorders while maintaining specialized care for severe conditions.



Depression and anxiety dominate

Together they represent the overwhelming majority of mental health DALYs (~1,381 per 100K combined).



Severe conditions carry disproportionate individual burden

Schizophrenia (210.2) and bipolar disorder (94.6) require maintained specialized care infrastructure.



Treatment exists but access and quality remain inadequate

Effective interventions are known, yet many people do not receive adequate care globally.



Stigma suppresses help-seeking and accurate data

Discomfort with disclosure means the true burden is likely higher than reported figures suggest.



Scale interventions for common disorders + maintain care for severe conditions

Policy must prioritize scaling evidence-based interventions while preserving specialized services.